

CUTANEOUS CHANGES COMMONLY ASSOCIATED WITH PREGNANCY

AT A GLANCE

Cutaneous changes result from the altered endocrine, metabolic, and immunologic milieu that characterize pregnancy.

Pigmentary disturbances, including hyperpigmentation, darkening of the linea alba, and melasma are the changes most commonly observed.

Significant change in nevi size is not a feature of most pregnancies.

Structural changes known to occur during pregnancy include, most commonly, striae distensae.

Pruritus is a common complaint during pregnancy and may be related to flare of a pre-existing dermatosis or onset of a specific dermatosis of pregnancy.

Melasma (Chloasma)

Melasma or chloasma is a common finding characterized by irregular, blotchy facial hyperpigmentation that occurs in up to 70 percent of pregnant women. This condition is exacerbated by sun exposure and oral contraceptive use in nonpregnant women. Melasma may regress postpartum, but it often persists, posing a therapeutic challenge.

Changes in Melanocytic Nevi

Historically, changes in melanocytic nevi have been considered normal during pregnancy. However, objective data are limited. Pennoyer et al. photographically monitored 129 melanocytic nevi in 22 healthy Caucasian women throughout pregnancy. Only eight nevi (6.2 percent) showed changes in

diameter between the first and third trimesters, with a mean size change of zero. The study concluded that significant change in nevus size—excluding nevi on the abdominal skin stretched by pregnancy—does not typically occur during gestation. Therefore, any pigmented lesion in a pregnant woman that changes in morphology (size, color, or shape) or develops symptoms (such as itching, bleeding, or scaling) should be considered for histopathologic evaluation.

Striae Distensae (Striae Gravidarum)

Striae distensae, commonly known as stretch marks, are the most frequent structural skin change during pregnancy. They typically appear on areas prone to stretching, including the abdomen, hips, buttocks, and breasts. Genetic factors—such as family history, personal history, and race—are the strongest predictors of striae development, outweighing factors like pregnancy weight gain or changes in body mass index. These findings suggest a strong genetic predisposition to this condition.

Pruritus in Pregnancy

Pruritus is a common symptom during pregnancy. It may be physiologic or may indicate a flare of a pre-existing skin condition (e.g., atopic dermatitis) or the onset of a pregnancy-specific dermatosis. Most pregnant patients with pruritic skin disease have conditions unrelated to pregnancy. However, a smaller subset develops dermatoses specific to pregnancy, some of which are associated with fetal risk.

DERMATOSES ASSOCIATED WITH FETAL RISK IN PREGNANCY

Pemphigoid Gestationis (Herpes Gestationis)

Pemphigoid gestationis (PG) is an intensely pruritic, vesiculobullous eruption that typically occurs in mid to late pregnancy or the immediate postpartum period. It classically presents during the second or third trimester with the sudden onset of severely itchy urticarial lesions on normal or erythematous skin.